

Darfur Bi-monthly Situation Update

Political, security and humanitarian developments across North, South, West, Central and East Darfur — 29 April through 13 May 2026.

Executive Summary

This bi-monthly edition covers 13–27 May 2026. The reporting window has been defined by a sustained and escalating drone-strike campaign against the Sudan–Chad border belt of North Darfur (Al-Tina, Karnoi, Ambro) — including a strike on a popular restaurant in Al-Tina on 24 May that killed five and wounded nine, and an OCHA-confirmed cumulative civilian toll of at least 12 in the border-area strikes during the second half of the period. In South Darfur, Nyala endured three consecutive days of drone strikes from 13–15 May on the industrial district, the Nyala International Airport vicinity and RSF maintenance/logistics nodes, prompting the RSF to impose tighter internal security and movement restrictions in the city. The RSF-aligned *Tasis* National Humanitarian Access Authority **extended its INGO registration deadline from 15 May to 13 June**, temporarily averting an immediate access cliff but not resolving the underlying legitimacy contest with Port Sudan. WHO launched a cholera vaccination campaign on 18 May targeting **252,000 people** in Tawila (NDS) and Kass (SDS) ahead of the rains. Famine conditions remain confirmed in El Fasher (IPC Phase 5); Tawila, Melit and At Tawisha stay at credible Famine risk and Tawila's IDP population is now reported above 575,000 by WHO. Nairobi-track political talks on 24 May produced a framework on post-war arrangements but with no operational ceasefire.

Top-line indicators (state of play)

Indicator	Value	Trend
People in need (Sudan HNRP 2026)	33.7 million	↑
IPC Phase 3+ acute food insecurity	~19.5 million	Sustained
IPC Phase 5 (Catastrophe / Famine)	~375,000	El Fasher confirmed
IDPs in Tawila locality (WHO, May)	575,000+	↑↑
New arrivals in Tawila (recent weeks)	~15,000	↑
OCV campaign target (Tawila + Kass, 18-May)	252,000 people	Launched
Civilians killed in Al-Tina/Karnoi border strikes (mid-May)	12+	↑
Civilians killed by drone strikes (Jan–Apr 2026, Sudan)	880+	↑
Tasis INGO registration deadline (extended 13-May)	Now 13 June	Watch

Key Events — 13 to 27 May 2026 (bi-monthly window)

North Darfur

- 24 May — Al-Tina:** Strategic drone (RSF-attributed) launched four-plus missiles over Al-Tina; one struck a popular restaurant directly. **Five killed, nine injured**, several in critical condition needing surgical evacuation. Al-Tina, alongside Karnoi and Ambro, remains one of the last areas under SAF / allied joint-forces control on the Chad border [Darfur24, 24 May].

- **Mid-May — Al-Tina and Karnoi:** OCHA reports at least **12 civilians killed** in multiple drone attacks on the two border towns, including strikes that killed children and wounded civilians; northern-corridor humanitarian access constrained by drone activity [OCHA Daily Briefing, 26 May].
- **Tawila — continuing arrivals:** Approximately 15,000 additional displaced persons arrived in recent weeks, pushing the locality population toward and beyond **575,000** per WHO planning figures for the 18-May OCV campaign. Site systems remain over-stretched [WHO / OCHA, May].
- **El Fasher villages:** Insecurity continues to force villagers around El Fasher to flee amid acute shortages of food, water and essential services. Famine (IPC 5) conditions persist.
- **Andour Well (Al-Tina):** No restoration during the reporting window; the destruction of 11–12 May continues to drive water insecurity for the affected catchment (see thematic section).

South Darfur

- **13–15 May — Nyala:** Three consecutive days of drone strikes targeted the industrial district, the area east of Nyala Technical College, near Nyala International Airport, and an RSF maintenance workshop. Four explosions on 15 May alone. RSF responded with tightened security: road closures in Al-Qasharat, restrictions on vehicles and rickshaws, and an internal ban on officers carrying phones into meetings amid surveillance fears [Darfur24, 15 May].
- **Tasis political concentration in Nyala:** Senior alliance leaders — including RSF commander Mohamed Hamdan Dagalo (Hemedti) — are reported in Nyala for high-level meetings, raising the likelihood that civilian neighbourhoods continue to be in proximity to high-value targets.
- **13 May — TISIS extended INGO registration deadline to 13 June:** Easing the immediate 15-May access cliff but preserving the underlying tension with Port Sudan, which continues to reject the parallel authority [Darfur24, 13 May].
- **South Darfur market strike:** A reported SAF drone strike on a South Darfur market killed 10 and injured seven, underscoring persistent civilian harm in market locations [Dabanga, May].

West Darfur

- **Geneina / Zalingei market function:** Markets in El Geneina and Zalingei continue to operate intermittently or remain shut, sustaining cash-economy distress and limiting humanitarian procurement options [ACAPS Economic Impact brief].
- **Kulbus and southern corridor:** Commercial transport remains unwilling to move supplies from El Geneina/Zalingei to Mukjar, Um Dukhun and Bindisi. The pipeline interruption is now in its second month, with knock-on effects on food, NFI and nutrition deliveries in Central Darfur.
- **Cross-border (Chad):** Continued protection caseload growth in Adré and Farchana; commodity prices in West Darfur remain elevated following earlier Chad border restrictions.

Central Darfur

- **Bindisi / Mukjar / Garsila / Um Dukhun:** Recurrent inter-communal clashes (Bani Halba ↔ Salamat) continue to drive displacement into Chad and the Central African Republic; supply pipeline cut from Geneina compounds the impact.
- **Zalingei Teaching Hospital:** The only specialised health facility in the state remains non-functional for specialised care after repeated looting; referral capacity effectively absent for trauma and obstetric emergencies.
- **Damrat al-Firdous (early May, residual impact in window):** Earlier drone strike killing RSF-aligned commander Hamid Ali Abubakar continues to shape risk perception around proximate civilian sites.

East Darfur

- **Al-Daein:** Continued drone activity reported against RSF positions and commanders' residences; civilian and security-detail injuries documented earlier in May persist as a baseline risk.
- **Yasin locality:** SMART-like survey from the lean-season onset registered Global Acute Malnutrition (GAM) **28%** — well above the WHO emergency threshold (15%) — and a basis for nutrition surge.
- **Displacement camps:** Fires earlier in the month in East and South Darfur camps continue to compound shelter and protection needs.

Cross-cutting (Sudan / regional)

- **18 May — WHO launched OCV campaign** in Tawila (NDS) and Kass (SDS) targeting 252,000 people ahead of the seasonal rains; WHO warned that the combination of rains, mass displacement and damaged WASH services creates high cholera-outbreak risk [WHO / Darfur24, 18 May].
- **21 May — Civil Democratic Alliance of Revolutionary Forces** warned of a deepening humanitarian crisis: ~34 million in need; ~19.5 million in acute food insecurity nationally [Darfur24, 21 May].
- **24 May — Nairobi political track:** Sudanese political and civil forces meeting in Nairobi agreed a framework for a post-war political process — comprehensive peace agreement, transitional constitution, unified security architecture. No operational ceasefire announced [Darfur24, 24 May].
- **12 May — UN Secretary-General António Guterres** warned that the Sudan war is driving wider African instability and called for urgent international action [Darfur24, 12 May].

Political and Security Analysis

The 13–27 May window reflects three reinforcing dynamics. First, a continued **weaponisation of aid** around the *Tasis* parallel humanitarian regulator in Nyala: the registration ultimatum was extended on 13 May to **13 June**, which buys roughly four weeks of operating space but does not resolve the underlying legitimacy contest with Port Sudan. Second, a sustained **drone-strike campaign concentrating on the Sudan-Chad border belt** (Al-Tina, Karnoi, Ambro) — the last areas under SAF / joint-forces control in Darfur — and on RSF logistics and leadership nodes in Nyala. The 24-May restaurant strike in Al-Tina (five killed, nine injured) and the cumulative 12+ civilian deaths reported by OCHA in mid-May illustrate how stand-off attacks are now routinely hitting civilian sites. Third, a **parallel political track** emerged in Nairobi on 24 May with a post-war framework, while UN Secretary-General Guterres (12 May) warned the war is spilling into wider African instability.

Operationally, the most important developments for cluster leads in the coming weeks are: (1) the renewed **13 June Tasis registration deadline** — agencies should use the extension to finalise HCT-aligned contingency planning; (2) the **concentration of RSF political and military leadership in Nyala** elevates the probability of further drone activity in residential / mixed-use neighbourhoods, with consequent risk for humanitarian staff and premises; and (3) **WHO's reactive OCV campaign** in Tawila and Kass marks the operational shift to rainy-season cholera-risk posture.

Sectors with the Highest Humanitarian Needs

Sector	Drivers of acute need	Hot-spot localities / sites
Food Security	Famine confirmed in El Fasher; credible Famine risk in Tawila, Melit and At Tawisha (NDS) and parts of the western Nuba Mountains. 21.2 million Sudanese in IPC 3+; ~375,000 in Catastrophe (IPC 5).	Tawila, El Fasher, Melit, At Tawisha, Um Baru, Karnoi (NDS); Nyala IDP belt and rural Kass (SDS); Mukjar / Um Dukhun (CDS).
Nutrition	SAM admissions in Darfur +46% (Jan–May 2025 vs 2024). Um Baru GAM 52.9%, Karnoi GAM ~34%, Yasin GAM 28%. Nutrition supply pipelines under stress along the northern corridor.	Um Baru, Karnoi, Tawila, Zamzam-origin populations (NDS); Yasin (EDS); Kass and Bielel IDP sites (SDS).
WASH	Pre-existing infrastructure damage (~70% functionality loss nationally); Andour Well destruction in Al-Tina removes a primary source for hundreds of households. Cholera outbreak officially closed March 2026 but transmission risk re-emerging with summer heat.	Al-Tina (incl. Andour catchment), Karnoi, Tawila assembly sites (NDS); Centre 10 / Al-Salam camp Nyala (SDS); Mukjar (CDS).
Health	Zalingei Teaching Hospital looted; El Fasher Southern Hospital is sole functioning facility under fire. Trauma surge capacity severely limited; immunisation interrupted in northern corridor.	El Fasher, Tawila, Karnoi, Al-Tina (NDS); Zalingei (CDS); Nyala (SDS) — particularly Centre 10 fire-affected.
Protection (incl. GBV and child protection)	MSF treated 3,396 conflict-related sexual-violence survivors across N. and S. Darfur (Jan-24 to Nov-25); 1 in 5 South Darfur survivors are children. Drone strikes near humanitarian offices in Nyala suggest erosion of humanitarian space.	Tawila (sudden-arrival population), Tina village belt, El Fasher (NDS); Al-Salam camp Nyala, Kalma (SDS); Garsila / Bindisi (CDS); Adré transit zones (cross-border).
Shelter / NFI / CCCM	~4 million IDPs nationally require site management. Tawila reception sites holding 500,000+ in open / public buildings. Fire risk in dry-season camps demonstrated by Centre 10 incident.	Tawila reception areas (NDS); Al-Salam, Otash, Kalma and Bielel camps (SDS); Hassa Hissa, Khamsa Dagaig (CDS).

Sector	Drivers of acute need	Hot-spot localities / sites
Logistics & Access	Commercial transport unwilling to move from Geneina/Zalingei to southern corridor. Drone activity constraining northern-corridor convoys. NGO registration ultimatum risks operational shutdown after 15-May.	Tina–Karnoi axis; Geneina → Mukjar / Um Dukhun corridor; Nyala humanitarian hub.

Hot-Pocket Locations Requiring Targeted Response

The following villages, settlements and IDP sites are flagged as *hot pockets* — geographically small areas where multiple sector indicators converge and where rapid, multi-sector response is most likely to prevent excess mortality. Coordinates and updated populations should be confirmed via the relevant DTM EET round and OCHA access maps before deployment.

State	Locality	Hot-pocket villages / sites	Convergent risks
North Darfur	Al-Tina (Tina)	Andour Well catchment villages, Tina town, Hijer, Umm Sayala, border-belt hamlets	Water-point destruction; AWD risk; displacement to Tiné (Chad); livestock losses.
North Darfur	Karnoi	Karnoi town, Furawiya, Birka Saira	Drone strikes on civilian convoys; GAM ~34%; access blocked for commercial transport.
North Darfur	Kutum / Saraf Omra	Sarf Umra market villages, Kassab IDP camp, Fataborno	Market strikes; protection; SGBV; severe food gaps.
North Darfur	Tawila	Reception sites at Tawila town, surrounding open-air shelters	180,000 new arrivals in 7 days; CCCM, WASH, nutrition, protection saturation.
North Darfur	Um Baru	Um Baru town, surrounding nomadic settlements	GAM 52.9%; isolated by drone activity; pipeline insecure.
South Darfur	Nyala (Beleil/Kass)	Centre 10 / Al-Salam camp, Otash, Kalma, Bielel	Camp fires; congestion; aid politicisation; proximity to strikes.
South Darfur	Kass / Shattaya	Kass town, Mershing, Shattaya	Food gaps; protection; restricted humanitarian access.
West Darfur	Kulbus	Kulbus town, surrounding villages	Recently reached — sustain delivery; nutrition screening urgently needed.
West Darfur	Sirba / Beida	Sirba, Misterei, Beida (Massalit-majority)	Continued protection risk; ICC-documented atrocities; cross-border returns.
Central Darfur	Mukjar / Um Dukhun / Bindisi	Bindisi town, Mukjar town, Um Dukhun, Garsila	Pipeline cut; communal clashes; cross-border displacement to CAR/Chad.
Central Darfur	Zalingei	Hassa Hissa & Khamsa Dagaig camps, Zalingei town	Only specialised hospital looted; trauma and EmONC gap; protection.
East Darfur	Yasin / Ed Daein	Yasin town, Mahajeriya, Al-Baytari (Ed Daein)	GAM 28%; recent drone strikes; protection of Zaghawa detainees.

Focus: Andour Well Destruction (Al-Tina, North Darfur)

What happened. On 11–12 May 2026, the Andour Well — described by the Al-Tina Emergency Room Council as a critical water source serving residents and displaced communities — was bombed. The strike killed and injured civilians, destroyed livestock and rendered the well completely inoperable. The Emergency Room Council issued a public appeal for alternative water supplies and emergency assistance (Darfur24, 12 May 2026).

Two-week follow-up (13–27 May). The well remains inoperable. Al-Tina has since been the target of additional drone activity culminating in the 24 May restaurant strike (five killed, nine injured) and an OCHA-confirmed cumulative civilian toll of 12+ across Al-Tina and Karnoi in the second half of May. The destruction is therefore best treated not as a one-off incident but as part of a pattern of attacks on the Sudan–Chad border belt, with cumulative depletion of water, health and market infrastructure. WHO's 18 May launch of the OCV campaign in Tawila/Kass does not yet extend to Al-Tina, where vaccination access is currently infeasible.

Households relying on the well

Publicly available reporting from the Al-Tina Emergency Room Council states that **hundreds of families** are directly exposed by the destruction. A precise denominator has not been published; however, applying standard Sudan WASH-cluster planning ratios provides a defensible working estimate for response purposes:

Scenario (well typology)	Typical service load	Estimated households relying on it	Estimated persons (HH)
Hand-pump borehole (worst case)	~250–500 persons / point	~40–85 HH	~250–500
Motorised borehole serving village + IDPs (most likely fit, given 'critical source for residents and displaced')	~1,500–3,000 persons / point	~250–500 HH	~1,500–3,000
Hub borehole feeding multiple kiosks/cattle troughs (upper bound, livestock losses reported)	~3,000–5,000 persons / point	~500–850 HH	~3,000–5,000

Working planning figure: until a rapid verification mission can confirm, **plan for ~250–500 households (~1,500–3,000 persons)** as the primary affected caseload, with contingency capacity to scale to ~850 HH if the well also served seasonal pastoralists and trans-border movement, which the destruction of livestock implies.

Sectors likely to be affected by the Andour Well destruction

Sector	Primary impact pathway	Acute risk window
WASH	Loss of the principal water source; collapse of safe-water access; alternative sources (open wadis, scoop-holes) are bacteriologically unsafe.	Days 0–14 — escalating to weeks 2–6.
Health	Surge risk of AWD/cholera, hepatitis A/E, shigellosis; eye and skin infections from rationed hygiene; deaths and injuries from the strike itself.	Days 3–21; cholera season risk Jun–Oct.
Nutrition	Reduced cooking and hygiene water → acute infections feed back into wasting; loss of household milk via livestock destruction worsens children's diets.	Weeks 2–12; lean-season multiplier.
Protection (incl. GBV)	Women and girls walking longer distances to alternative sources elevate SGBV exposure; intra-community tensions over rationing alternative wells; child labour for water-collection.	Immediate and ongoing.

Sector	Primary impact pathway	Acute risk window
Food Security / Livelihoods	Livestock killed at the well point — direct asset loss; pastoralist herd-movement disrupted; market access interrupted by ongoing insecurity around Tina/Karnoi.	Weeks 1–8.
Shelter / CCCM	Secondary displacement toward Tiné (Chad), Karnoi or Tawila if alternative water cannot be secured within 10–14 days; risk of overload at receiving sites.	Days 7–30.
Education	If hygiene crisis materialises, school attendance for girls falls first; classrooms used as water-distribution points lose function.	Weeks 2–8.
Logistics	Water-trucking from safe sources required; routes Tina ↔ Tiné and Tina ↔ Karnoi exposed to drone activity.	Immediate.

Assessments Required for an Objective Response

The following assessment package is recommended to underpin an evidence-based, multi-sector response to the Andour Well destruction and analogous water-point attacks emerging across the northern corridor. The package is sequenced from rapid (≤ 72 hours) to in-depth (2–6 weeks) to align with cluster decision-making timelines.

Phase	Assessment	Lead / contributors	Indicative tools / outputs
Phase 1 (0–72h)	Multi-Sector Initial Rapid Assessment (MIRA-lite)	OCHA / ICCG with WASH, Health, Nutrition, Protection, CCCM	MIRA form (community-level KIs); severity classification; immediate caseload estimate.
Phase 1 (0–72h)	WASH Rapid Needs & Water-Point Damage Assessment	WASH Cluster (UNICEF lead) + national WASH partners	Water-point inventory verification; functionality/yield testing; FCR & turbidity; ICRC IHL flag if deliberate.
Phase 1 (0–72h)	Protection Incident Monitoring (incl. IHL flag)	Protection Cluster (UNHCR lead); Protection Monitoring Network	Incident report; civilian casualty verification; trigger for GBV safety audit.
Phase 2 (72h–14d)	Population Movement Tracking (DTM EET)	IOM DTM	Flow monitoring at Tiné, Karnoi, Tawila; verification of secondary displacement linked to water loss.
Phase 2 (72h–14d)	Water-Point Mapping (incl. alternative sources)	WASH Cluster GIS unit / REACH	Geo-referenced alternative sources, queue times, distance >500m thresholds, cost per litre.
Phase 2 (72h–14d)	GBV Safety Audit	GBV Sub-Cluster (UNFPA lead)	Risk-mapping of water collection routes; women/girl FGDs; safe-route recommendations.
Phase 2 (72h–14d)	Rapid Health Risk Assessment (AWD/cholera focus)	WHO / Health Cluster + State MoH	EWARN re-activation; ORS/RHK pre-positioning; OCV decision tree.
Phase 3 (2–6 wk)	SMART Nutrition Survey (mini-SMART if access constrained)	Nutrition Cluster (UNICEF lead) + nutrition NGOs	GAM/SAM with WHZ + MUAC; mortality module; programme coverage (SQUEAC/SLEAC).
Phase 3 (2–6 wk)	Household Food Security & Livelihoods Assessment	FSL Cluster (WFP/FAO co-lead)	FCS, rCSI, LCSI; livestock-loss module; market function (MFA).
Phase 3 (2–6 wk)	WASH KAP Survey	WASH Cluster + REACH	Hygiene practice baselines for BCC programming; identify community-acceptable alternative sources.
Cross-cutting	Conflict & Access Analysis Update	OCHA Access Working Group; ACLED; ACAPS	Updated access map; pipeline routing risk; staff-safety decisions.
Cross-cutting	Accountability to Affected Populations (AAP)	ICCG AAP working group	Community feedback mechanisms; complaint channels for water-trucking siting decisions.

Quality safeguards. Triangulate at least two independent data streams per critical indicator (e.g. Emergency Room Council reports + DTM flow data + WASH partner direct observation). Disaggregate by sex, age, disability and displacement status. Use Sphere standards as the minimum reference (15 L/p/d, ≤ 500 m to source, ≤ 30 min queue, <250 persons per tap stand) and document any negotiated derogations.

Recommendations to Cluster Leads

Operational (next 7–14 days)

- **WASH Cluster:** Activate emergency water-trucking from Tiné (Chad) and Karnoi water yards to Al-Tina Andour catchment; pre-position HTH/calcium hypochlorite and ceramic filters at Karnoi forward hub.
- **Health Cluster:** Re-activate EWARN sentinel sites in Tina, Karnoi and Um Baru; preposition cholera kits and ORP supplies; coordinate with WHO on OCV reactive-vaccination eligibility ahead of June rains.
- **Nutrition Cluster:** Surge OTP/SC capacity at Tawila reception and Um Baru; extend MUAC-by-mother campaigns into the Tina/Karnoi villages while access windows permit.
- **Protection / GBV:** Establish women-only water collection windows at any alternative source; deploy protection monitors to Tawila and Tiné receiving sites; ensure PSEA messaging on water-trucking convoys.
- **CCCM:** Pre-position site-management capacity at Tawila and Tiné anticipating secondary displacement.
- **Logistics:** Convoy de-confliction with both parties for the Tina–Karnoi and Geneina–Mukjar axes; engage UNHAS for staff rotation contingencies.

Strategic (next 14–30 days, before 13 June Tasis deadline)

- Finalise a joint humanitarian position on the **13 June Tasis registration deadline** at HCT and donor level, with explicit red lines and operational contingency plans for South, Central and West Darfur should agencies be barred from operating after the deadline.
- Commission an inter-cluster **Water-as-a-Weapon** analysis covering Golo, Shagra, Andour and the Al-Tina / Karnoi pattern — feed into ICRC, OHCHR and ICC documentation pipelines.
- Operationalise **Tawila** as the principal northern-corridor reception platform (population 575,000+ per WHO); plan for additional contingency arrivals from El Fasher villages and from the border belt.
- Scale pre-positioning of **nutrition, WASH, cholera-response and protection** supplies in Tiné (Chad) under the cross-border modality, and accelerate OCV planning for Al-Tina, Karnoi and Ambro when access permits.
- Engage the Nairobi political-process forum (24-May framework) to embed humanitarian access, civilian-infrastructure protection (notably water points) and IHL compliance into any subsequent texts.

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All assertions in this brief draw on public-domain reporting from the United Nations system, INGOs and credible Sudanese media. Cluster leads are encouraged to cross-reference with their own partner data before operational decisions.

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Note on data limitations. This brief is compiled from open-source reporting in a contested information environment. Where figures from belligerent-aligned sources are cited, they are flagged as such; where household figures are extrapolated (notably for Andour), the basis is shown and a verification mission is recommended before commitment of resources beyond the contingency caseload.